

CANONICAL LESSON

Referrals That Do Not Lose the Patient

Decision-centred foundations for accountable sector intelligence

Production Unit	PU-B07-C30
Notebook	V7_B_N10
Status	Controlled educational draft v0.1
Data boundary	Synthetic instructional data only

Central decision: Design referral intelligence that preserves continuity, acknowledgement, urgency, capacity, handoff, closure, privacy, and clinical authority.

Learning outcomes

Why this matters

A referral recorded as sent can still lose the patient when receipt, follow-up, and closure are not verified.

The analytical workflow is useful only when evidence, uncertainty, authority, and action are connected explicitly. The model supports professional judgement; it does not replace accountable institutional authority.

Worked cross-sector example

An urgent referral remains open until the receiving service acknowledges capacity and records a governed outcome.

Boundary: The example supports learning and review. It does not constitute an official statistic, eligibility determination, enforcement action, clinical judgement, or operational authorization.

Practice and reflection

- 1.
- Add network edges.
- 2.
- Separate urgent cases.
- 3.

- Protect privacy.

Answer key

- 1.
- Build directed sending-receiving flows with counts, delays, capacity, and completion.
- 2.
- Stratify by authorized urgency definition and case mix; never infer urgency from delay alone.
- 3.
- Aggregate routes, minimize identifiers, control access, log use, and suppress small flows.

Operational readiness checklist

- Purpose and decision owner are named.
- Evidence definitions, provenance, and lawful access are documented.
- Uncertainty and abstention are visible.
- Human review, escalation, monitoring, incident response, and retirement are assigned.
- Equity, accessibility, security, and sector-specific approval are complete.